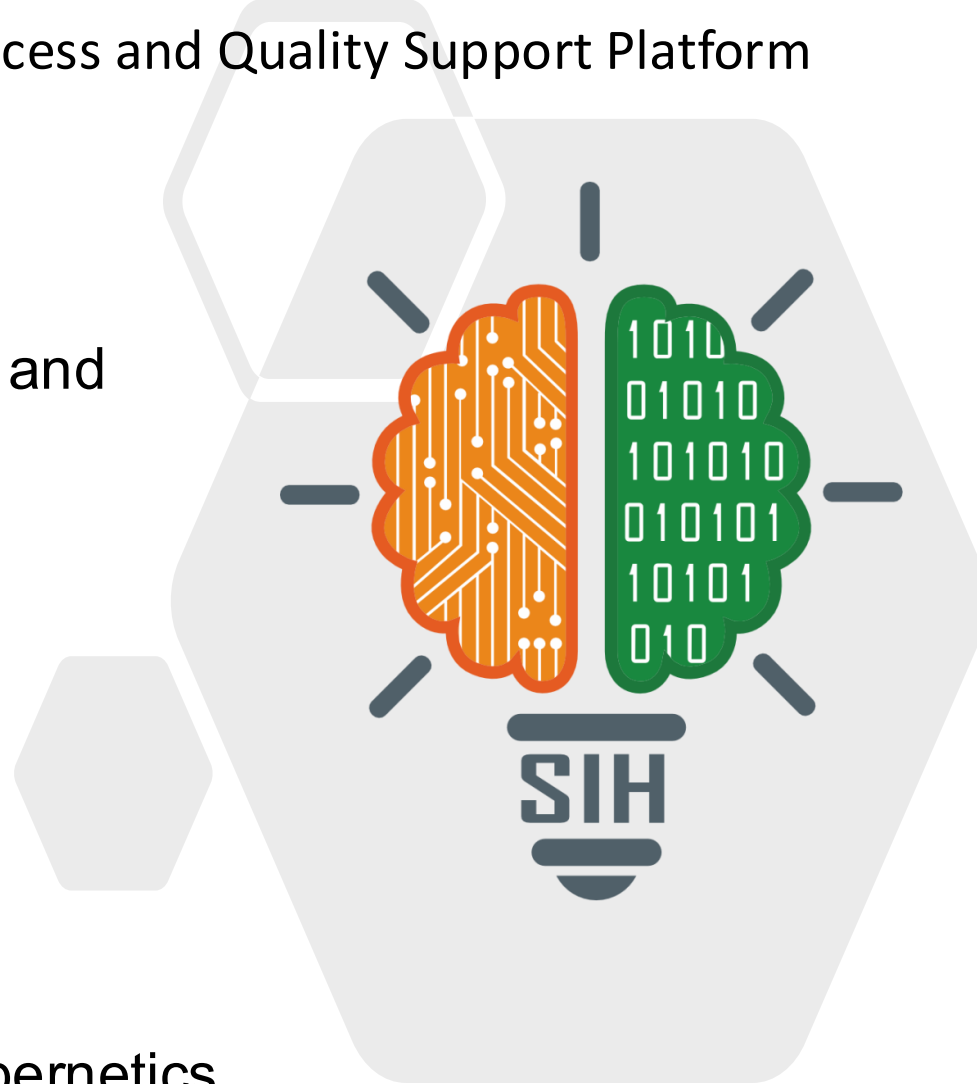


SMART INDIA HACKATHON 2026



SevaSetu: Integrated Care-Access and Quality Support Platform

- **Problem Statement ID** – 26133
- **Problem Statement Title** – Accessibility and quality of public healthcare services.
- **Theme** - HealthTech
- **PS Category**- Software
- **Team ID** - SIH26133
- **Team Name(Registered on portal):** Cybernetics



SevaSetu

Integrated Care-Access and Quality Support Platform

Detailed explanation of the proposed solution:

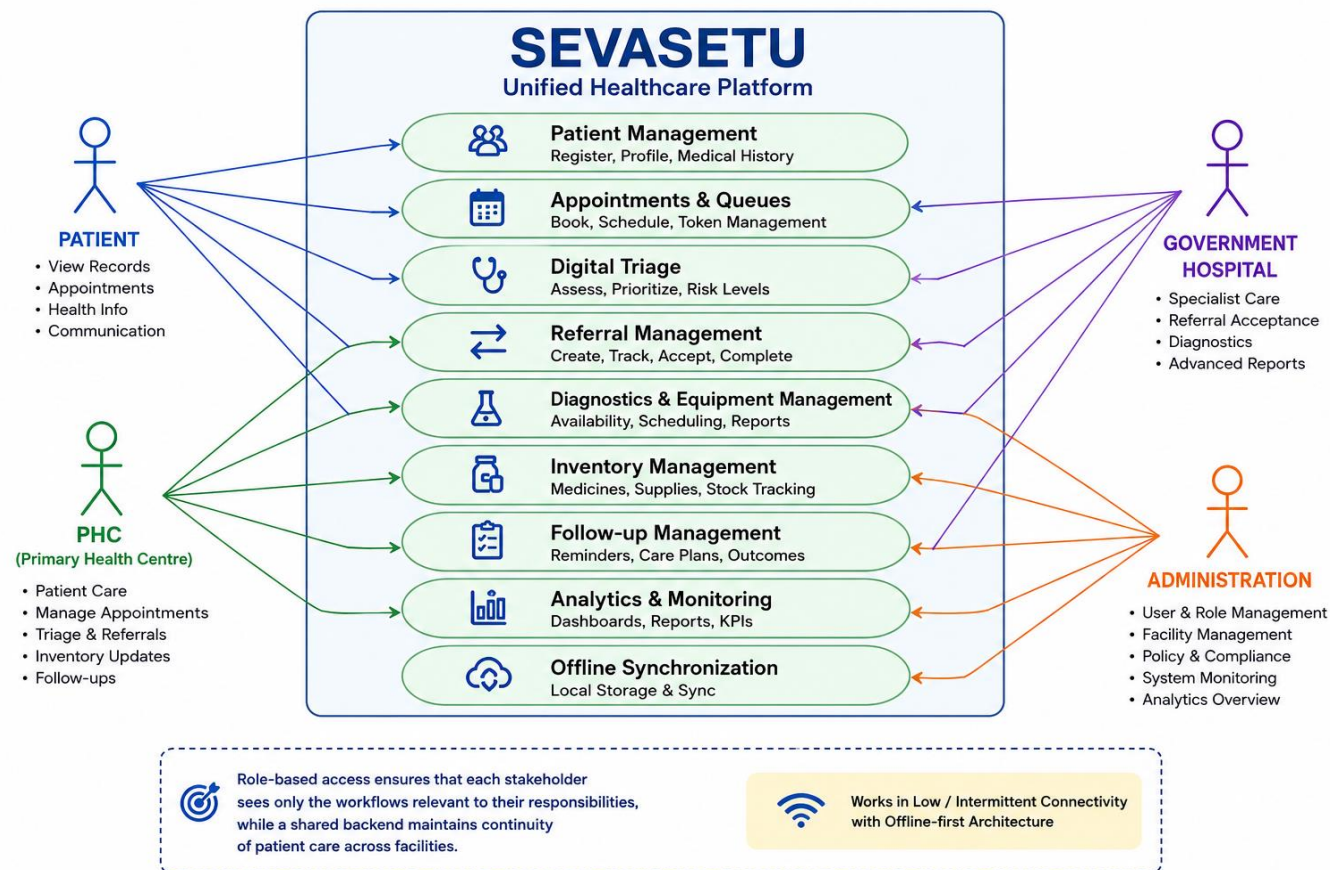
- Patient/ASHA offline-first PWA — booking, digital triage, teleconsultation, multilingual UI. **[Hackathon MVP]**
- Doctor/PHC dashboard — live queue, longitudinal records, teleconsult console, one-click referrals. **[Hackathon MVP]**
- District admin dashboard — facility map, inventory tracking, high-risk follow-up, analytics.

How it addresses the problem:

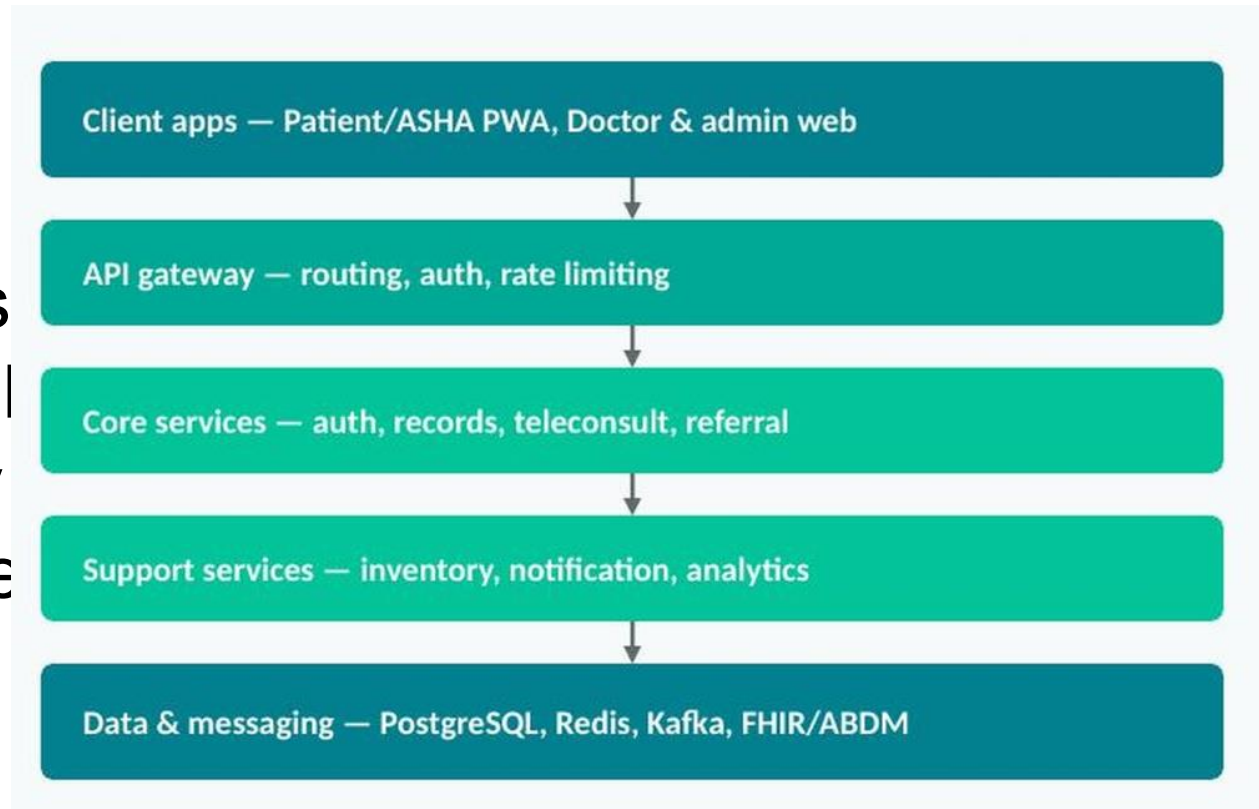
- Unified FHIR-based record ends fragmentation across sub-centre → PHC → district hospital
- Teleconsultation + facility-matcher closes travel-distance and delayed-referral gaps
- Offline-first design + multilingual UI solves connectivity and health-literacy barriers.

Innovation and uniqueness:

- **Store & Forward Tele-health:** Captures patient data completely offline and auto-syncs to doctors when the network returns.
- **Smart Facility Routing:** Checks live doctor and equipment availability *before* creating a referral to prevent wasted travel.
- **Offline Triage & ABDM Native:** Calculates patient risk (Red/Yellow/Green) locally on device and integrates smoothly with national ABHA IDs.



- Technologies frameworks, |
- Methodology Charts/Image



ages,
Flow

RISKS AND MITIGATION

- **AI triage errors**
AI used for decision support only, never autonomous diagnosis
- **Data privacy and security**
RBAC, end-to-end encryption, secure authentication
- **Offline sync conflicts**
Timestamped, versioned syncs with unique operation IDs
- **Low user adoption**
Multilingual, role-specific interfaces cut the learning curve
- **Interoperability**
Standards-based APIs for integration with govt. health systems

Viability of the solution

- **Cost-effective:** open-source stack and lightweight infra cut deployment cost by an estimated 60–70% vs. proprietary systems.
- **Simple adoption:** multilingual, role-based UI minimizes training time for ASHA and frontline workers
- **Lower operational load:** digitized appointments, referrals, queues and follow-ups cut manual admin work
- **Reaches low-connectivity areas:** offline-first design keeps essential services running without reliable internet.

Parameter	Existing Solutions (e.g., eSanjeevani)	SevaSetu (Proposed)
Setup Cost	High (Requires steady internet/hardware)	Low (Runs on basic smartphones)
Connectivity	Live Video (Fails in 2G)	Offline-First (Store & Forward)
Scalability	Manual routing	Automated Smart Facility Routing

Reduced travel and waiting



Patients view appointments, referrals and diagnostic queues before visiting facilities.

Improved referral completion



End-to-end referral tracking reduces delays and missed referrals.

Continuity of care



Longitudinal records keep patient information accessible across PHCs and government hospitals.

Better resource utilization



Real-time visibility of medicines, equipment, queues and workforce improves facility management.

Faster triage and care

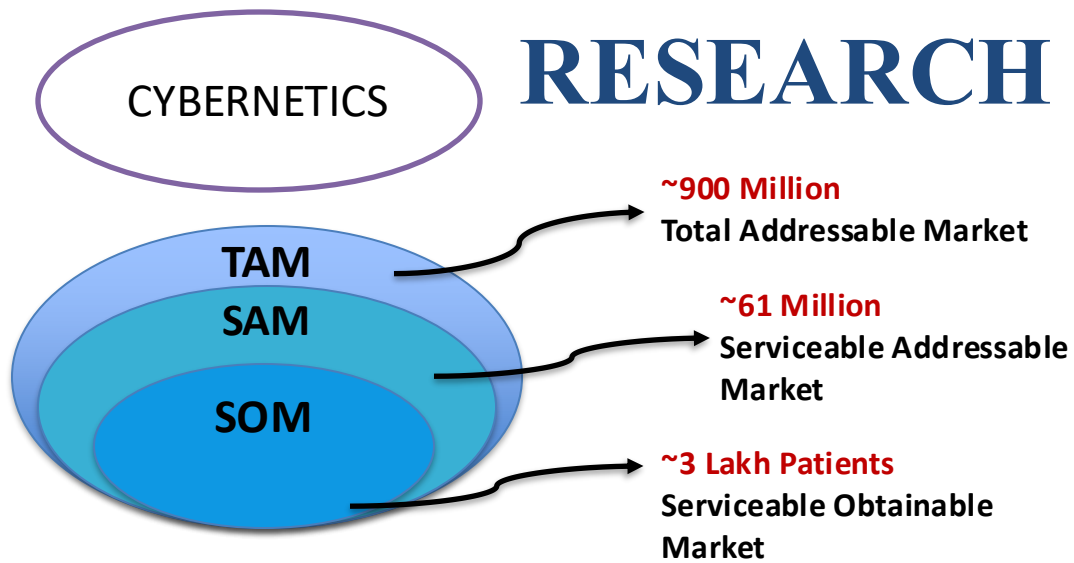


AI-assisted triage helps prioritize high-risk cases and guide appropriate care pathways.

Improved accessibility



Offline-first operation extends reliable healthcare support to low-connectivity regions.



TAM-Total Addressable Market (Indian Rural Healthcare)

The entire Indian rural population facing severe doctor shortages, connectivity issues, and fragmented public healthcare access.

SAM-Serviceable Addressable Market (Maharashtra State Focus)

Targeting Maharashtra's rural population currently dependent on primary health centers (PHCs) and sub-centers for initial diagnosis.

SOM-Serviceable Obtainable Market (Year 1 Initial Adoption)

Pilot rollout across 5 underserved districts. We expect to reach 3 Lakh+ patients via ASHA workers assuming a 5% early adoption rate.



REFERENCE

- **Government Benchmarks:** eSanjeevani (Hub-and-spoke model, but heavily dependent on live video) & **ABDM Sandbox** (National digital health backbone & ABHA IDs).
- **Private Competitors Analyzed:** Practo / Tata 1mg (Urban/semi-urban focus) and **Tattvan e-Clinic** (Closest rural franchise competitor, but lacks offline-first autonomy).
- **Technical Standards & Links:**
 - **HL7 FHIR Specification:** For interoperable health records (hl7.org/fhir).
 - **W3C PWA Standards:** For Offline-first sync & IndexedDB storage.
 - **Web Demo:**

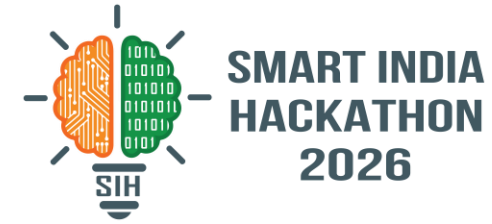


LINKS

🔗 IMPORTANT RESEARCH & REFERENCE LINKS

- **ABDM API Sandbox (Govt Integration):** sandbox.abdm.gov.in
- **ABDM FHIR Profiles (India Standards):** nrces.in/ndhm/fhir/r4/
- **eSanjeevani (Market Benchmark):** esanjeevani.mohfw.gov.in
- **W3C PWA Offline Specs:** developer.mozilla.org/en-US/docs/Web/Progressive_web_apps
- **SevaSetu Prototype Code:**

IMPORTANT INSTRUCTIONS



Please ensure below pointers are met while submitting the Idea PPT:

1. Kindly keep the maximum slides limit up to six **(6)**. (Including the title slide)
2. Try to avoid paragraphs and post your idea in points /diagrams / Infographics /pictures
3. Keep your explanation precise and easy to understand
4. Idea should be unique and novel.
5. You can only use provided template for making the PPT without changing the idea details pointers (mentioned in previous slides).
6. You need to save the file in PDF and upload the same on portal. No PPT, Word Doc or any other format will be supported.

Note - You can delete this slide (Important Pointers) when you upload the details of your idea on SIH portal.